14.4 Neurogenic Shock*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective *No specific complaint* **Objective** • Cervical spinal cord injury • BP 82/48 mmHg, HR 48/min • Warm, dry skin below the injury	**ACTUAL** Ineffective peripheral tissue perfusion related to loss of sympathetic vascular tone as evidenced by hypotension, bradycardia and warm, dry skin below the level of injury.	• MAP within the target set by the physician.	• Keep the spine immobilized. • Give fluids and vasopressors as prescribed. • Give atropine for symptomatic bradycardia as ordered. • Keep ECG and BP on continuous monitoring.	• Met when MAP is on target.
Subjective *No specific complaint* **Objective** • Weak cough • Vital capacity reduced	**ACTUAL** Ineffective breathing pattern related to respiratory muscle weakness as evidenced by reduced vital capacity.	• Ventilation adequate, vital capacity stable and SpO_2 on target.	• Follow vital capacity, SpO_2 and ABG. • Prepare for ventilation if breathing worsens. • Perform assisted coughing as taught.	• Achieved while ventilation is adequate.
Subjective *No specific complaint* **Objective** • Cannot regulate temperature below the injury	**RISK** Risk for decreased body temperature as evidenced by risk factors: loss of sympathetic control below the level of injury.	• Core temperature 36–37.5 °C.	• Measure temperature every 2–4 hours. • Adjust room temperature and bed covers. • Keep him covered during procedures.	• Outcome reached when temperature is normal.

*References for this care plan: [2], [35], [37], [128]. Full entries are listed in the References section of the CPMN book.